

1. Administrative & Study Identification

Full Study Title: A Phase III, Multicenter, Double-Blind, Placebo-Controlled, Treat-Through Study to Assess the Efficacy and Safety of Induction and Maintenance Therapy With R07790121 in Patients With Moderately to Severely Active Crohn's Disease **Short Title/Acronym:** SIBERITE-1 **Protocol Number:** GA45331 (EU Trial CTIS Number: 2024-513053-69-00) **NCT Number:** NCT06819878 **Sponsor Name:** F. Hoffmann-La Roche AG / Genentech **Investigational Product:** Afimkibart (also known as R07790121, PF-06480605, RVT-3101) - Anti-TL1A monoclonal antibody **Phase of Development:** Phase III **Medical Monitor:** Sponsor Medical Monitor (Contact: global-roche-genentech-trials@gene.com)

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the efficacy and safety of induction and maintenance therapy with afimkibart (R07790121) compared to a placebo in participants with moderately to severely active Crohn's disease (CD). **Secondary Objectives:** To assess the percentage of participants achieving clinical remission, endoscopic response, symptomatic remission, ulcer-free endoscopy, corticosteroid-free clinical remission, and the maintenance of clinical and endoscopic remission through Week 52.

2.2 Background & Rationale Crohn's disease (CD) is a type of inflammatory bowel disease causing chronic inflammation of the digestive tract. For many people living with CD, symptoms do not improve even with conventional treatments (corticosteroids, biologics, small molecules), indicating a high unmet medical need for treatments with better benefit-risk profiles. Afimkibart (R07790121) is a fully human neutralizing monoclonal antibody targeting tumor necrosis factor-like ligand 1A (TL1A), which plays a central role in the regulation of gut mucosal immunity and fibrotic pathways involved in the pathogenesis of IBD.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Placebo-controlled **Blinding:** Double-blind **Randomization:** Randomized (2:1 ratio for study drug versus placebo)

3.2 Planned Enrollment & Duration Target \$N\$: 600 participants **Number of Sites:** Approximately 355 locations globally (including the US, France, Brazil, Argentina, etc.) **Estimated Study Start:** 17-Mar-2025 **Estimated Primary Completion:** 31-Dec-2028

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age 16 to 80 years. 2. Confirmed diagnosis of moderately to severely active Crohn's disease (CD). 3. Body weight ≥ 40 kg. 4. Demonstrated inadequate response, loss of response, and/or intolerance to at least one protocol-specified conventional therapy (aminosalicylates, corticosteroids, immunosuppressants) or advanced CD therapy (biologics or targeted small molecules like anti-TNF, anti-IL-12/IL-23, JAK inhibitors).

4.2 Exclusion Criteria 1. Current diagnosis of ulcerative colitis (UC), indeterminate colitis, microscopic colitis, ischemic colitis, infectious colitis, or radiation colitis. 2. Participant with a history of ≥ 3 bowel resections or diagnosis of short gut/short bowel syndrome. 3. Presence of an ileostomy, colostomy, or ileoanal pouch. 4. Presence of symptomatic bowel strictures, fulminant colitis, toxic megacolon, abdominal/perianal abscess, or current diagnosis/suspicion of primary sclerosing cholangitis.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: - **Induction Phase (12 weeks):** Study drug given on Day 1, Week 2, and then every 4 weeks until Week 10. - **Maintenance Phase (40 weeks):** Study drug given every 4 weeks from Week 12 to Week 52 (at a dose of either 450 mg or 150 mg). **Route of Administration:** Intravenous (IV) infusion during the induction phase, followed by Subcutaneous (SC) injection during the maintenance phase. **Duration of Treatment:** 52 weeks (Treat-through design), followed by an optional Open-Label Extension (OLE) phase.

5.2 Concomitant & Prohibited Therapy Permitted: Stable background conventional CD therapies may be permitted according to study guidelines. **Prohibited:** Concurrent treatment with advanced biologics, JAK inhibitors, or other targeted small molecules used for CD outside the study protocol.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
1	-35 to 0	Informed Consent, Medical History, Confirmation of Eligibility, Baseline Endoscopy
2	0 to 12	Randomization, IV Infusions (Day 1, Wk 2, Wk 6, Wk 10), Safety/Efficacy Assessments
3	12 to 52	SC Injections every 4 weeks (Q4W), Clinical scoring, Endoscopy at Wk 52
4	52+	End of Study, Transition to optional Open-Label Extension (OLE) or 12-week Safety Follow-up visits (Weeks 6 & 12 post-last dose)

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: 1. Percentage of Participants with Clinical Remission per Crohn's Disease Activity Index (CDAI) Score ≤ 150 at Week 52. 2. Percentage of Participants with Endoscopic Response (decrease in Simple Endoscopic Score for CD [SES-CD] of $\geq 50\%$ from baseline) at Week 52. **Measurement Method:** Clinical assessment for CDAI; Central/Local Reading of Ileocolonoscopy for SES-CD.

7.2 Secondary & Exploratory Endpoints - Percentage of participants with symptomatic remission. - Percentage of participants with ulcer-free endoscopy. - Average of Daily Number of Liquid or Very Soft Stools in the Past Week (SF) and Daily Abdominal Pain Scores (APS). - Percentage of participants with corticosteroid-free clinical remission. - Maintenance of clinical and endoscopic remission at Week 52.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection of AEs/SAEs, vital signs, physical examinations, and laboratory assessments throughout the 52-week treat-through period and OLE. **Serious Adverse Events (SAEs):** Continuously reported as per GCP and local regulatory guidelines. **Specific Safety Monitoring:** Monitoring for infection risks (including Tuberculosis, C. difficile, CMV, Hepatitis B/C, and HIV).

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) for primary and secondary efficacy evaluations; Safety Set for all participants who receive at least one dose of the investigational product. **Sample Size Justification:** $N=600$, designed to provide adequate power to detect clinically meaningful differences between the afimkibart and placebo arms across primary induction and maintenance endpoints. **Handling of Missing Data:** Participants who discontinue study treatment prematurely or require prohibited rescue medications will generally be considered non-responders (e.g., Non-Responder Imputation) for primary categorical endpoints.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Regular onsite and remote monitoring visits to evaluate protocol adherence, safety reporting, and data integrity. **Audit Trail:** Clinical data will be entered into

validated electronic Case Report Forms (eCRFs) maintaining full audit trails.

11. Study Identifiers & Governance

Primary ID: GA45331 **Registry Number:** NCT06819878 **Sponsor/Lead Organization:** F. Hoffmann-La Roche AG **Study Title:** SIBERITE-1 (Afimkibart in Moderately to Severely Active Crohn's Disease) **Official Regulatory Title:** A Phase III, Multicenter, Double-Blind, Placebo-Controlled, Treat-Through Study to Assess the Efficacy and Safety of Induction and Maintenance Therapy With R07790121 in Patients With Moderately to Severely Active Crohn's Disease

12. Clinical Framework

Primary Condition: Moderately to Severely Active Crohn's Disease (CD) **Diagnosis Threshold:** Confirmed diagnosis of CD with moderate to severe activity; prior failure/intolerance of conventional or advanced therapies. **Medical Methodology:** Anti-TL1A monoclonal antibody biologic therapy **Optimization Target:** Clinical remission (CDAI $\leq$ 150) and Endoscopic response/remission.

13. Study Procedures & Methodology

Management Pathway: Step-up induction therapy followed by maintenance therapy for active CD. **Education Protocol (HCP):** Investigator protocol training, endoscopic central reader training, and adverse event reporting instruction. **Education Protocol (Patient):** Informed consent, instructions on self-reporting symptoms (CDAI components like stool frequency and abdominal pain score) via patient diaries. **Quality Audit Mechanism:** Independent Data Monitoring Committee (IDMC), centralized endoscopic readings.

14. Observation & Follow-Up Schedule

Total Duration: 52 weeks (plus up to 35 days screening, optional open-label extension, and 12-week safety follow-up) **Onsite Visit Cadence:** Frequent visits during the 12-week induction phase, followed by approximately every 4 weeks (Q4W) during the maintenance phase. **Remote Monitoring Frequency:** Between site visits as necessary via subject diaries. **Checklist Review Interval:** Routine reviews aligned with every 4-week dosing schedule.

15. Sign-off & Approval

Role	Name	Signature	Date		:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]					
Statistician	[Name]	_____	[DD-MMM-YYYY]					